coursera

Depressive and Bipolar Disorders

Today's goal • 3/3 items

Module 1

Learn With PsycLearn Essentials

Start Your Course Here

- ✓ A Welcome Video From Your Instructor
  Video • 2 min
- ✓ Welcome to PsycLearn Essentials
  Reading • 3 min
- ○ Survey: What Would You Like to See Next?
  Reading • 2 min
- ✓ How to Earn Certificates
  Reading • 2 min

Course Overview

- ✓ What's in Your Course
  Reading • 5 min

Study Success in PsycLearn

- ✓ Study Tips for Success in PsycLearn Essentials
  Reading • 5 min

Additional Course Information

- ✓ Additional Information
  Reading • 10 min
- ✓ Representation in Research
  Reading • 1 min

Module 2

Opening to Depressive and Bipolar Disorders

Lesson 1: Culture and Mood Disorders

- ✓ Focus of the Course
  Reading • 2 min
- ✓ Classifying Psychopathology
  Reading • 4 min
- ✓ Culture and Psychopathology
  Reading • 4 min
- ✓ Culture and Intersectionality
  Reading • 4 min
- ✓ Culture and Stigma
  Reading • 4 min
- ○ Check Your Understanding: Culture and Mood Disorders
  Practice Assignment • 5 min

Lesson 2: Introduction to Depressive and Bipolar Disorders

- ✓ Depressive and Bipolar Disorders
  Video • 6 min
- ○ **Living with a Mood Disorder: A Case Study**
  Reading • 10 min
- ○ Overview of Depressive and Bipolar Disorders
  Reading • 10 min
- ○ Check Your Understanding: Introduction to Depressive and Bipolar Disorders
  Practice Assignment • 5 min

Key Takeaways

- ○ Key Takeaways: Opening to Depressive and Bipolar Disorders
  Reading • 10 min

Module 2 Quiz

- ○ Module Quiz: Opening to Depressive and Bipolar Disorders
  Graded Assignment • 10 min

Module 3

Depressive and Bipolar Disorders

Module 4

Biological Contributing Factors

Module 5

Psychological Contributing Factors

Module 6

Treatments for Depressive and Bipolar Disorders

Module 7

Suicide

Module 8

Conclusion

Module 9

Cumulative Quiz

Module 10

Course Resources

Module 11

PsycLearn Essentials APA Student Resources

Living with a Mood Disorder: A Case Study

0:04 / 3:00 1x

Recall the case study about Sonia who was struggling with mental health issues, her gender identity, and acceptance within her family and community. Recognition of her need for help began with a visit to her school counselor, who saw Sonia's distress and encouraged her to take the first steps toward treatment. For a different perspective on Sonia's story, consider the therapist who began working with Sonia on referral from Sonia's school counselor.

> “
>
> With permission from Sonia, I can share with you some information from our conversations about her mental health struggles. Sonia was referred by her college counselor as a vulnerable student who was at risk of dropping out of college and spiraling into a mental health crisis. Although she was hesitant and not initially trusting, we eventually developed a rapport and I began seeing her as a patient.
>
> Sonia's early years were affected by several conditions that identified her as a member of a marginalized group. These included barriers from language, culture, and economics. As the child of an immigrant family, her elders had no experience understanding what resources were available to them or navigating the bureaucracy that could help them succeed in their new home country. As an adolescent, she simply floated in life with little direction, but providence must have played a role in putting her in the local community college, where she met a counselor that recognized her need for help.
>
> Sonia's life history identifies her as a member of a marginalized population, but within our sessions, she also revealed that she identified as a transgender woman, which is not accepted by her family and community. Compounding these conditions were the symptoms that Sonia described about her mental health. Within her culture, mental health issues are viewed as arising from a lack of self-control or an unwillingness to adhere to cultural values.
>
> Within our sessions, Sonia described symptoms that included feelings of alienation, lethargy, difficulty concentrating, feeling hopeless and worthless, and thoughts of self-harm. These are symptoms that could identify her as someone with a depressive disorder. However, Sonia also described periodic episodes where she would remain awake for days, had racing thoughts, felt invincible, had boundless energy, and would act impulsively. These later symptoms can be described as mania. Taken together, this combination of depression and mania suggest that a proper diagnosis would be a form of bipolar disorder.
>
> Working together, we developed a treatment plan that included both biological and psychological treatments for her symptoms. She's expressed some reservations about the need for medications, but admits to recognizing the benefits of a balanced mood, the ability to concentrate on her life goals, and grounding to continue her journey for acceptance with her family and community about her gender identity.
>
> ”

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